RxSense: launching claim-time rules like PGx without surprises

Prepared by Vinod Kumar G for RxSense, October 2026

What I understood

In February 2026 Blue Genes announced a partnership to build pharmacogenomics (PGx) into the RxSense enterprise platform, so that genetic insight informs medication selection at the point of adjudication [1]. RxSense offers Core, Modular and Full-Service tiers, and Full-Service covers plan design and network configuration [2]. Your AI engineering team is building golden datasets and regression harnesses wired into CI/CD [3].

The opportunity

Every new claim-time rule ships into plan configurations that are already live. Two things have to hold: the rule changes only the claims it is meant to change, and the lookup it adds doesn't slow a real-time claim. I don't know how RxSense tests this today. My suggestion is to replay a golden set of claims through every rule or configuration release, compare the outcomes rule by rule, and block the release on any change it didn't declare.

What it could be worth

About USD 17,000 to 320,000 a year in engineering and QA time. That excludes faster launches and avoided production incidents, which I can't price from outside.

- Formula: releases a year × manual test hours per release × share saved × hourly cost.
- Low: 6 × 80 h × 50% × $70. High: 20 × 200 h × 80% × $100.
- Every input is my assumption, because RxSense publishes no volumes. Your real numbers would replace them in a first conversation.

What I built to test the idea

claim-replay (https://github.com/VinodKumarGgithub/claim-replay) uses 5,000 synthetic claims and a demo engine that returns NCPDP reject codes. It adds a CYP2C19 rule for clopidogrel, which posts a soft message and still pays the claim. The demo release also contains a typical slip: a metformin quantity limit edited by mistake.

- The replay blocked that release: 196 claims changed through a rule the release didn't declare, and the 12 intended changes were accepted.
- The PGx rule's p95 was 7.1 ms against a 15 ms budget. Claims for other drugs never called the genotype lookup.
- It runs as a CI gate, and one interface points it at a staging endpoint.

A pilot

Three weeks on one rule type or one client configuration, using synthetic or de-identified claims, in staging only, alongside one of your engineers. We would measure before and after: manual test hours per release, defects caught before release, p95 latency per rule, and days from configuration ready to live.

Why me

- I lead a 12-member team on a PBM platform handling 1M+ transactions a day for 11 enterprise clients.
- I cut response times of critical PBM transaction APIs by 90%+, from several seconds to under 100 ms.
- I automated POC transaction testing with a BullMQ/Redis pipeline: 600 hours of manual work became 6–12 hours.

Sources

1. https://www.santelog.com/actualites-sante-nasdaq/blue-genes-partners-rxsense-deliver-actionable-pgx-point-adjudication
2. https://www.rxsense.com
3. https://job-boards.greenhouse.io/rxsense/jobs/4713834005

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